

THE FEDERATION OF ASIA-OCEANIA PERINATAL SOCIETIES



The FAOPS Funiculus

Promoting Science & Art of Perinatology

ISSUE HIGHLIGHT

The Effect of COVID-19 Pandemic on the Delivery of Basic Maternal and Newborn Health Services in Luzon, Philippines

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Perinatal CME Programme in Bangladesh

The Perinatal Society of Bangladesh

Report of the 21st FAOPS Congress 2022

The Perinatal Society of Malaysia

A Neonate Narrates

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My Mother's Home

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From the Editor's Desk

Dear Readers,

Funiculus refers to the umbilical cord arising from the placental bed nourishing the baby, it also refers to the stalk arising from the ovaries of several plants, hence a reminder of our roots and the importance of disseminating the tree of knowledge. The FAOPS Funiculus therefore aims to fulfil one of the aims in the formation of the Federation of Asia Oceania Perinatal Societies or FAOPS, that is to promote the science, and the art of perinatology in the region of Asia-Oceania. As we meet in Tokyo, Japan for the 22nd FAOPS congress and celebrate our 45th year of formation, we hope that this will serve as a memento to our Asia-Oceania perinatal fraternity of the struggles experienced, in ensuring the good health of our mothers and babies across the globe. We are still facing the sequelae of the COVID-19 pandemic, an era which had taught us many important lessons, from which we hope we have learned. It is indeed timely for us in the fraternity to think of the future, revisit our goals and ensure that no mother or baby is left behind in our pursuit to ensure their good health and survival. With this, the first edition of the FAOPS Funiculus highlights the effort made by our colleagues in the Philippines who had evaluated the impact COVID-19 had on the World Health Organization's Basic Emergency Obstetric and Newborn Care (BEmONC), a service protocol for emergency obstetric functions at primary health care level adopted in Luzon, the Philippines. Included in this edition are activities conducted by the Perinatal Society of Bangladesh and the 21st FAOPS Congress report. We also hope you will enjoy the two narrations of perinatal struggles which will enable us to reflect and better appreciate our patients. The publications committee would also like to announce the finalization of our FAOPS' research topic publication in the Frontiers of Surgery journal, which can be read via the link <https://www.frontiersin.org/research-topics/34624/saving-mothers-and-babies-for-the-new-world>. We wish to sincerely thank the current FAOPS President, Professor Samuel Rajadurai, our FAOPS' past president Professor Satoshi Kusuda, and all council members and country representatives who have motivated the publication of this issue.

Enjoy The FAOPS Funiculus!


The Editor



The Objectives of FAOPS

The Federation of Asia and Oceania Perinatal Societies (FAOPS) is a federation of national societies of perinatal and neonatal medicine in the geographical region of Asia and Oceania. It was established in 1978 with the following federation objectives:

1. To promote the science and art of Perinatology.
2. To promote maternal welfare.
3. To promote fetal and neonatal well-being.
4. To maintain a liaison with other organizations or professions involved in Perinatology.
5. To promote cooperation and goodwill with international bodies involved in Perinatology.
6. To provide expert advice to governmental and other bodies pertaining to Perinatology.
7. To promote research and training in Perinatology.
8. To support country perinatal society guidelines on the ethical practice of the profession.



The Bye-Law Objectives

1. To promote, encourage and assist endeavours in all fields related to Perinatology.
2. To disseminate and communicate information on knowledge and skills concerning Perinatology, particularly those that present problems peculiar to the region.
3. To improve and uphold the highest standards of care in Perinatology.
4. To recommend common policies in these matters when requested by National Societies whenever joint action can or should be pursued in such matters.
5. To publish a Journal or to coordinate with other official Journals related to Perinatology.
6. To award scholarships, professorial lectures and prizes as determined from time to time.
7. To disseminate information relating to the objectives of the Federation to National Bodies, International Agencies, Foundations, and individuals with the Aim of furthering the Federation's interests.
8. To seek assistance, financial or otherwise, from National Bodies, International Agencies, Foundations or any other relevant party on matters of significance in maternal, perinatal and neonatal health.
9. To acquire, maintain and dispose of any assets to further the activities of the Federation.
10. To encourage and promote research and training in fields related to Perinatology.

The Effect of the COVID-19 Pandemic on the Delivery of Basic Maternal and Newborn Health Services in Luzon, Philippines

By Maria Stephanie Fay S. Cagayan, MD, PhD

President, Perinatal Association of the Philippines

Background

Even before the COVID-19 pandemic, maternal health has already been a global health concern with the United Nations committing to solve this through the Millenium Development Goal ⁵, and then through the Sustainable Development Goal ³. In pursuit of these, the Philippine Department of Health (DOH) adopted the World Health Organization's Basic Emergency Obstetric and Newborn Care (BEmONC) in 2009. BEmONC was a service protocol for emergency obstetric functions at primary health care level. BEmONC facilities were created by improving rural health units (RHU) providing pregnancy and postnatal care. This resulted in increased rates of skilled and facility-based deliveries from 2008 to 2015.¹

However, the start of the COVID-19 pandemic affected delivery of maternal services. Some studies have shown that maternal and birth outcomes have worsened during the pandemic, with increased rates of poor reproductive outcomes such as maternal deaths, preterm births, and maternal depression.²⁻⁴ Utilization of maternity services has also been adversely affected, with hospital births decreasing³ and home deliveries increasing.^{5,6} The disruption of healthcare services due to the pandemic has harmed the well-being of pregnant individuals, with factors such as fear of infection, lockdowns, burnout of healthcare providers, reduced availability of drugs and supplies, and diversion of resources to treat COVID-19 patients contributing to the reduction in attendance and availability of reproductive, maternal, and neonatal health services.⁷⁻⁹ This study evaluated the impact of the pandemic on BEmONC services in Luzon, the largest island group in the Philippines.

Methods

From February to May 2021, 244 facilities evenly distributed among eight regions in Luzon were surveyed (Table 1). The facilities were selected using a two-stage method, with provinces and

government-run BEmONC facilities being chosen. To assess the impact of the pandemic on maternal and neonatal care, the study looked at the number of monthly deliveries in each facility from December 2019 to November 2020, as well as the availability of health facility services. A checklist was used to assess the facilities' available resources. Focus group discussions (FGDs) were also conducted either via Zoom or face to face with 200 stakeholders, including 102 health personnel and 98 community members. Eight to twelve focus group discussion participants per region were selected through snowball referral sampling, beginning with a list of local government units (LGU) and BEmONC staff and community members provided by the municipal/city health officer and/or by the DOH.

Table 1. Provinces per Region in Luzon Surveyed Based on Maternal Mortality Ratio (MMR) strata

Region	Province/s	Number of RHUs	MMR stratum
I	La Union	10	High
	Pangasinan	10	Medium
	Ilocos Sur	10	Low
II	Cagayan	10	High
	Nueva Vizcaya	10	Medium
	Quirino/Batanes	11	Low
CAR	Apayao/Abra	11	High
	Benguet	10	Medium
	Mountain Province	10	Low
III	Aurora/Zambales	11	High
	Bulacan	11	Medium
	Pampanga	10	Low
NCR	District 2	10	High
	District 1	9	Medium
	District 3	10	Low
CALABARZON	Rizal	11	High
	Cavite	10	Medium
	Quezon	10	Low
MIMAROPA	Romblon	10	High

	Palawan	11	Medium
	Mindoro Oriental	10	Low
V	Catanduanes	10	High
	Albay	10	Medium
	Sorsogon	9	Low

Results and Discussion

Most facilities were reported as functional. Looking closely though, only one-third of the facilities had emergency medicines available and half had a complete team of BEmONC-trained professionals. Out of the 244 facilities surveyed, nearly all had access to electricity and running water. However, some renovations took too long to complete hence, some spaces were eventually used for COVID-19 response instead. (Figure 1). Moreover, there was an observed increase in the number of deliveries immediately after the Enhanced Community Quarantine (ECQ) was imposed. From around 1600 deliveries a month, the number jumped to approximately 1900. This rate was sustained until the end of the period covered. No maternal mortality was noted in the facilities that were surveyed but there was 1 intrauterine death in 1 facility. The fetal heartbeat was already absent at the time of initial consult and the mother had no prenatal consults due to the lockdown.

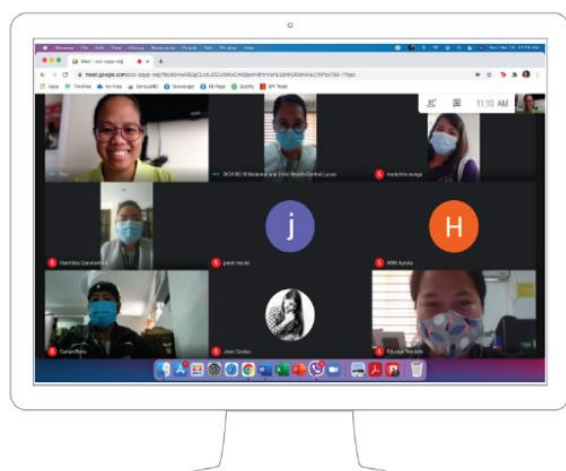
Responses from the FGD show that the COVID-19 pandemic was seen both as a barrier and a facilitator of BEmONC (Figure 2). Some patients preferred obtaining maternal services from BEmONC facilities as these were seen as a safer option than hospitals that were used as COVID-19 facilities. There was an increase in patients who are consulting in maternity clinics and RHUs. However, since the patients were also wary of going out, there were imminent deliveries. Concurrently, there were difficulties in referral and transportation to CEmONC facilities of complicated maternal cases and these cases were handled in RHUs instead.

To offer ambulance services, some RHUs had to be resourceful by using tricycles or their staff's private vehicles. RHUs would also pool together pregnant women with similar ultrasound schedules and drive them to the laboratory. This initiative was affected by the directive of the Department of Transportation lessening the allowed number of passengers in a vehicle during the ECQ. Some RHUs do not have ambulance vehicles for BEmONC purposes at all.

Figure 1. BEmONC facility survey



Figure 2: Focused group discussions (FGD) with stakeholders held via Zoom(A), and face-to-face (B).



A



B

Numbers of available BEmONC staff were seen to decrease. Some were inflicted with COVID-19, others were isolated for some time due to first-hand exposure to COVID-19 patients, and several were reassigned to COVID-19 areas. Skills training and capacity building for potential BEmONC staff was also stopped.

These findings highlight the decrease in BEmONC services delivery. This was evident in some

regions where 17% of BEmONC facilities were not operational during the survey and even before the pandemic. This might have discouraged patients from using BEmONC facilities. The FGDs also confirmed these findings, revealing that insufficient healthcare workers, emergency vehicles, beds, and facilities were among the obstacles that contributed to the reduced provision of BEmONC services.

However, patients' preference for BEmONC facilities over hospitals highlights the need for strict adherence to COVID-19 protocols to protect the staff and the patients. Identifying a BEmONC facility as a source of transmission could lead to closure and impede service provision. Moreover, while BEmONC can help decongest CEmONC facilities, it may overwhelm low-capacity facilities.

Conclusion

The effects of the COVID-19 pandemic and government response to BEmONC services varied. Despite the lockdown's expected adverse impact on BEmONC, those in Luzon continued to provide services and also saw an increase in usage in half of the regions. The pandemic resulted in a reallocation of resources and staff, as well as restricted access to healthcare facilities for non-COVID cases. However, the diffusion of patients from referral centers to community-based peripheral facilities led to a rise in demand and utilization of BEmONC services.

Recommendations

BEmONC capacity should be improved. Inclusive adjustments in service delivery involving community health workers during a pandemic are also needed.

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Perinatal CME Programme in Bangladesh

By the Perinatal Society of Bangladesh

The Bangladesh Perinatal Society (BPS) organized a CME on “Thyroid Insufficiency in Pregnancy and Perinatal Outcome” on 5th March, 2023, in the Hospital Conference Room of Shaheed Suhrawardy Medical College, a tertiary level referral Hospital in Dhaka city. This was a part of periodic educational program of BPS which was chaired by Professor Laila Arjumand Banu, President of Bangladesh Perinatal Society and Former Secretary General of FAOPS. Prof. T A Chowdhury, Founder President and Advisor of BPS, Prof. Nazmun Nahar, Advisor of Bangladesh Neonatal Forum and Principal, Shaheed Suhrawardy Medical College adorned the program as chief guests. The session was glorified by Professor Mohammad Shahidullah, Vice President of BPS and Former President of FAOPS. President of Obstetrical and Gynaecological Society of Bangladesh, Director of Directorate General of Health Services & Line Director, Maternal, Newborn, Child & Adolescent Health and Director of Shaheed Suhrawardy Medical College who were present as special guests in this program.

A good number of senior teachers and doctors of different positions including undergraduate and postgraduate students from OBGYN, Pediatrics and Neonatology from host medical college and other medical colleges and institutions enriched the program by their cordial presence. The program started with recitation from Holy Quran followed by welcome address by senior professors from OBGYN, Shaheed Suhrawardy Medical College.

There were 2 speakers. Assistant professor Dr. Shahnaz Parveen Joba from department of OBGYN, Shaheed Suhrawardy Medical College presented the 1st part of the session and her talk was on “Thyroid Insufficiency in Pregnancy: an Overview”. The 2nd part was presented by Associate Professor Dr. Sadeka Choudhury Moni from department of Neonatology, Bangabandhu Sheikh Mujib Medical University (BSMMU) and she talked on “Screening for congenital hypothyroidism: why and when? Evidences and recommendation”. Both the speakers nicely described this important health problem in pregnancy, its effect on the developing fetus and the causes of congenital hypothyroidism, limitations of symptom based diagnosis of congenital hypothyroidism and thereby the need for a screening program to diagnose early and prevent intellectual disability related to congenital hypothyroidism.

The presentations were followed by question answer session and discussion on the topic which was moderated by Professor Md Abdul Mannan, Secretary General of BPS and Deputy Secretary General, Western Region of FAOPS. Group of panelists comprised by Senior Professors of OBGYN and Neonatology expressed their valuable comments on the questions raised by the audiences. At the end of the scientific session special guests, chief guests and chairperson of the program delivered their valuable comments on the presentation along with focus and directives on identification and management issues. The expert panel also urged to play significant role by the physicians involved in perinatal care for successful implementation of newborn screening program adopted by ministry of health and family welfare, Bangladesh.

Figures: The Bangladesh Perinatal Society's CME Programme



Saving Mothers and Babies for the New World – A Report of the 21st FAOPS Congress 2022

By the Perinatal Society of Malaysia

The Federation of Asia & Oceania Perinatal Societies (FAOPS) and the Perinatal Society of Malaysia (PSM) have successfully organised the 21st Congress of the FAOPS & 28th Congress of the PSM from 25th to 28th August 2022. Embracing the theme “Saving Mothers & Babies for the New World” this was the first-ever virtual conference held by FAOPS in the uncertain times of COVID-19 pandemic that struck the world unprecedentedly.

Aims of the congress

The theme of the congress “Saving Mothers & Babies for the New World” provides the backbones for the following aims:

- ◆ Provide basic knowledge of resuscitating mothers and newborns, antenatal care, delivery room and neonatal intensive care practices.
- ◆ Updates of perinatal healthcare policies.
- ◆ Updates on state-of-the-art and ground-breaking science that enhance the outcomes of mothers and babies.
- ◆ Sharing of experience in the management of perinatal care in the region.

FAOPS Organising & Scientific Committee Members

The virtual congress was planned meticulously; with Dr TP Baskaran as the Chair of the Organising Committee and Associate Professor Dr Azanna Ahmad Kamar as the Chair of FAOPS Scientific Committee. The Scientific Committee comprised of experts from the member countries including the President of the FAOPS, Professor Satoshi Kusuda (Japan), Professor Mamoru Tanaka (Japan), Professor Ranjan Pejaver (India) and Professor Ju Lee Oei (Australia). Numerous virtual meetings were held to ensure an attractive program for the delegates in upholding the theme.

Virtual platform

Neudimenxion Sdn Bhd was responsible in providing the FAOPS 2022 virtual platform. The virtual platform was beautifully designed with easy navigation during the congress. The audience was able to participate in parallel tracks of live and pre-recorded webinars, virtual workshops, and experienced visiting virtual booths. The videos-on-demand were made available to all delegates for 3 months after the congress.

The Programme and The Participants

The 21st Congress of the FAOPS & 28th Congress of the PSM offered interesting and excellent scientific program. The virtual congress was officiated by Malaysia Deputy Minister of Health, YB Dato Dr Haji Noor Azmi Bin Ghazali and graced by PSM patron Her Highness YTM Raja Dato Seri Eleena Bt Almarhum Sultan Azlan Muhibuddin Shah A-Maghfur-lah.

There were total of 5 virtual workshops, 2 hands-on workshops, 3 plenary lectures and 20 symposia. In addition, there were 4 industry symposia and a session on experience sharing in the management of perinatal COVID-19. Seventy-one speakers were invited for the main congress and 39 speakers for the workshops. The speakers were experts in their fields and research and delivered excellent scientific presentations for the delegates.

A total of 543 participants attended the virtual main congress. Majority were from Malaysia followed by Japan and Indonesia. The workshops received great reviews by the participants. The titles of the workshop include below:

- 💧 Rescue Strategies & Care of the Surgical Neonate
- 💧 Perinatal Pathology – Learning from the Loss
- 💧 Fetal Growth Essentials & Antenatal Surveillance
- 💧 Quality Improvement : The Basics
- 💧 Root Cause Analysis
- 💧 Advanced Twins Ultrasound Workshop (Hands-On)
- 💧 Advanced Neonatal Airway and Ventilation Workshop (Hands-On)

Awards

The following were the winners of the respective categories :

Best Neonatal Oral Paper Presentation	Dr Yusuke Noguchi (Department of Pediatrics and Developmental Biology, Tokyo Medical and Dental University)	Gene expression profile analysis of umbilical cord mesenchymal stem cells revealed fetal programming due to chorioamnionitis
Best Obstetrics Oral Paper Presentation	Dr Kiichiro Furuya (Department of Obstetrics and Gynecology, Rinku General Medical Centre, Osaka Japan)	The impact of Covid-19 on progression of labour: a single-institution case series report
Best Poster Presentation	Ms Vra Sashti Tharanni Ravindran (Faculty of Medicine, University Malaya, Malaysia)	The incidence of SARS-CoV-2 vertical transmission in University Malaya Medical Centre

Industry participation & sponsorship

We are proud to have 15 renowned international and local industry companies that have kindly participated in this congress.

The 21st Congress of the FAOPS & 28th Congress of the PSM was a successful virtual congress. We expressed our appreciation to all FAOPS country members as well the delegates for their support in this virtual congress.

Page | 13

A Neonate Narrates

By Ranjan Pejaver, Professor of Neonatology, Chief Neonatologist, People tree@ Meenakshi Hospital, Bangalore, India.

Hurray! I have made it.

From the wet, watery, restricted womb to this big, bright world. Mind you, it was not easy. I was not actually prepared for this. I was taken by surprise. It was that fateful moment, suddenly I saw a gaping opening in the roof of my chamber. The water around me suddenly drained. It is a horrible experience! Have you ever been in a bathtub full of water, soaped and without your knowledge, all the water has run out? Then two green heads with face half covered, peered down the hole. Do they want to hide their identity, lest I may take revenge? Before I could blink, two rubberized hands grabbed me, pulled me out and severed my lifeline. What is more, one of them held me upside down and was raising the hand, as if to spank me. Just then, out of sheer fright, reflexly I gave a loud shriek. Funnily enough, this seems to have made all around heave a sigh of relief!



Anyway, I was spared the bash, instead, passed on to another green person who placed me on a green towel in the corner of the room. Strangely enough, though it was a shock to me, I believe people around had planned and arranged this calculated mission of dislodging me from my private chamber.

As I was celebrating my escape from spanking, a tube was shoved into my nostril. I spluttered, sneezed, and cried in protest, which stopped them from continuing this assault. Next was the act of rubbing me with a green dry cloth. I enjoyed it, it was exhilarating! I thought that life was going to be fun. Sorry, just then I heard somebody saying, "he is three weeks early, we have to keep a close eye on him." I felt like a convict who had escaped from prison. Just then I started feeling this heat from

above. Good gracious, it was a heater coil above me. Instead of a baby boy, I felt more like a chicken burger on the barbecue!

I was then shifted to a room and put in a glass case. Have you ever slept inside an empty aquarium? This was mean. But worse was yet to come. The people around me, most of them in green/blue gowns, did all sort of nasty things like shoving tubes up and down my various orifices. I was tired. I wanted to be left alone to take a nap. This I thought they did when nothing happened for a few moments. Then came this young lady with a needle. She may be pretty at other times, but at that moment she looked scared, and nervous even more than me! She poked me three times before she could take some blood and shove what they call a cannula into me. I was wondering whether she was one of Dracula's assistants and how often was she going to do this to me.

It was time for some food. They brought a bottle of milk. The smell was very appetizing. Believe me, as I was priming my taste buds, they shoved a tube down my nostril and down the milk went without any of it touching my lips and tongue. This is the height of cruelty. That too, the amount given was few spoonfuls! Stingy stinkers. I looked around the room. There were quite a few in these glass cabinets. O.K. I am a small fellow, but there were one or two chubby, sumo wrestlers' type. Why didn't they fight this imprisonment?

There were a few who were quite shriveled.

The veteran across was appearing to say,
"You fella, if you think that being a neonate is all breast milk and honey, wait till you see everything. The fun has just begun."



My Mother's Home

By Azanna Ahmad Kamar, Associate Professor of Neonatology, University Malaya, Kuala Lumpur, Malaysia



In my mother's home I was fairly still
My legs tried to kick but they wouldn't move
In my mother's home I thrived yet grew still
Nourished by the placenta and motherly love

In my mother's home, swallow I could not
The water surrounding becoming more not standing tear
From my mother's home I could always hear my mother
But the water was too much, I had to be cut out from my lair

From my mother's home I moved to a place so cold so
hollow
Where swallow I could not as pain seared through my throat
In the hollow place was where I felt the bellows
Drumming air through my chest in imperfect notes

In the hollow place I was fairly still
My legs tried to kick, yet move they could not
In the hollow place I thrived yet grew still
Nourished by gushing fluid through the veins of my hand
So sore were my hands yet move they would not

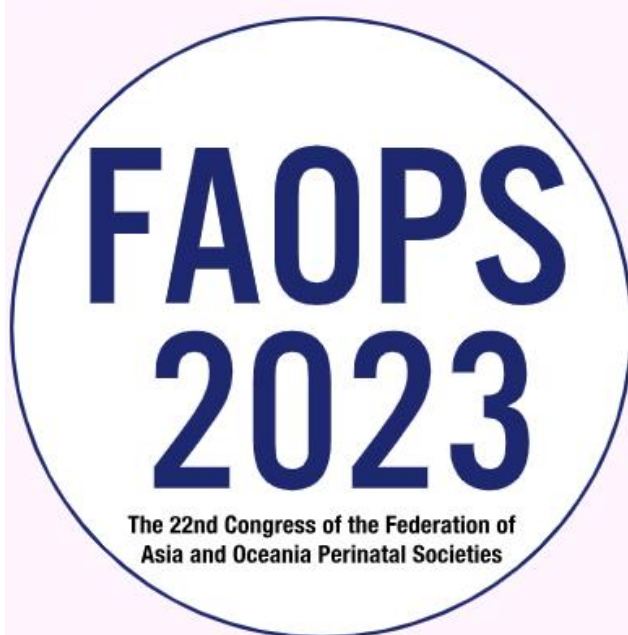
In the hollow place I could not suck I could not swallow
My tongue quivered as milk streamed through plastic
ripping tears down my throat
In the hollow place I could not always hear my mother
For days and months, simply noises of the bellows
Of the alarm monitors, in imperfect notes

In the hollow place I wished for my mother's home
But for days and months I was asked, to continue with the
flow

The choice is not mine, I was told not to moan
This hollow place is my destined home
My home to grow with the flow, to swallow the sorrow
It is indeed the home I know.



SDGs for Maternal and Neonatal health across Asia-Oceanian region



Congress Chair

Mamoru Tanaka

Professor, Department of Obstetrics and Gynecology,
Keio University School of Medicine

Venue

Keio Plaza Hotel Tokyo

**7TH-9TH
OCTOBER, 2023**



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